



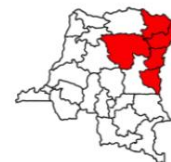
EBOLA PRESIDENTIAL FORCE TASK 17

Ministry of Public Health, Hygiene and Social Welfare

National Institute of Public Health

Public Health Emergency Operations Center

MVE17 Incident Management System



Situation Report of the 17th Ebola Virus Disease Outbreak / DRC

SitRep N°115/MVEBDB/07/09/2026

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📅 Publication date: September 7, 2026



HIGHLIGHTS (last 24 hours)

As of September 6, 2026, 82 new confirmed cases of Ebola Virus Disease (EVD), including 36 community deaths, were recorded in the provinces of Ituri (39 cases), North Kivu (39 cases), and Haut-Uélé (4). However, 15 deaths intra-CTE were recorded. No new health zones have been affected in the last 24 hours.

ACCUMULATIVE CASE 6,686	ACCUMULATIVE DEATH CONFIRMED 3,226	LETHALITY 48.3%	PATIENTS IN ISOLATION/CTE 819	ACCUMULATIVE HEALED 1,563	TRACKING RATES CONTACTS (Of the Day) 85.3%
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Affected Provinces and Affected Health Zones



6 Provinces Affected

Haut-Uélé, Ituri, North Kivu,
South Kivu, Tshopo & Bas Uélé



61 out of 151
health zones affected
(40.4%)



Summary of key points from the last 24 hours

As of September 6, 2026, **there were 82 new confirmed cases** of Ebola Virus Disease (EVD), including **36 community deaths**. These cases are distributed in the provinces of **Ituri (39 cases), North Kivu (39) and Haut-Uélé (4)**. In addition, **15 Intra-CTE deaths** were reported, including 12 in Ituri, 2 in North Kivu, and 1 in Haut-Kivu. Uélé, bringing **the number of deaths for the day to 51**.

In the past 24 hours, **15 patients** have been declared cured after obtaining two negative results for Ebola virus disease (11 in Ituri, 3 in Haut Uélé and 1 in North Kivu).

From the start of the epidemic until September 5, 2026, **there were 6,686 confirmed cases, including 3,226 deaths**, representing a case fatality rate of **48.3 %**. The epicenter remains the Ituri province, accounting for **80.2%** of confirmed cases recorded in the country and **47.6%** of the new cases.

No new health zones have been affected in the last 24 hours.

hours. Thus, **61 out of 151 health zones (40.4%)** remain affected in the 6 affected provinces. Ituri with 28 out of 36 health zones affected, followed by North Kivu (16/34), Haut-Uélé (6/13), Tshopo (7/23), South Kivu (1/34) and Bas-Uélé (3/11).

Table 1. Distribution of confirmed cases and deaths by affected province, as of September 6, 2026.

Province	New confirmed cases (24h)	Confirmed cases	Deaths (confirmed)	Lethality	Affected health zones
Ituri	39	5365	2,426	45.2%	28/36 (77.8%)
North Kivu	39	1039	681	65.5%	16/34 (47.0%)
Haut-Uélé	4	253	106	41.9%	6/13 (46.1%)
Tshopo	0	22	9	40.9%	7/23 (30.4%)
South Kivu	0	3	1	33.3%	1/34 (2.9%)
Bas Uélé	0	4	3	75.0%	3/11 (27.3%)
Total	82	6,686	3,226	48.3%	61/151 (40.4%)

Table 2. Distribution of confirmed cases and deaths by province and health zone, as of September 6, 2026.

Province / Health Zone	Cumulative number			Current situation (24 hours)			
	Case (n)	Deaths (n)	Lethality	New Case	Community deaths (Swab+)	Confirmed deaths within the CTE	Total confirmed deaths
Ituri	5365	2426	45.2%	39	16	12	28
Adja	11	1	9.1%				0
Aru	8	6	75.0%				0
Ariwara	8	2	25.0%				0
Aungba	12	5	41.7%				0
Bamboo	176	39	22.2%	5	3		3
Boga	2	2	100.0%				0
Bunia	1488	464	31.2%	5	2		2
Damascus	30	9	30.0%				0
Drodoro	15	7	46.7%				0
Fataki	78	30	38.5%	1	0		0
Gethy	7	3	42.9%	1	1		1
Kambala	2	0	0.0%				0
Kilo	33	12	36.4%				0
Komanda	113	75	66.4%	6	5		5
Lita	242	121	50.0%	5	0		0
Logo	13	5	38.5%				0
Lolwa	25	7	28.0%				0
Mahagi	4	1	25.0%				0
Mambasa	17	7	41.2%				0
Mandima	41	23	56.1%	3	2		2
Mangala	290	141	48.6%	1	1		1
Mongbwalu	634	290	45.7%	2	0		0
Nia-Nia	226	115	50.9%				0
Nizi	707	273	38.6%	7	1		1
Nyankunde	125	36	28.8%	1	1		1
Rimba	11	4	36.4%				0

Rwampara	987	345	35.0%	2	0		0
Tchomia	60	25	41.7%				0
To ventilate	N / A	378	N / A	0	0	12	12
North Kivu	1039	681	65.5%	39	20	2	22
Blessed	176	132	75.0%	13	10	1	11
Good	9	4	44.4%	2	1	1	2
Butembo	208	144	69.2%	4	2		2
Goma	1	0	0.0%				0
Kalunguta	18	9	50.0%	1	1		1
Katwa	457	296	64.8%	12	5		5
Kayna	1	0	0.0%				
Kyondo	23	15	65.2%	2	0		0
Lubero	6	4	66.7%				0
Mabalako	10	7	70.0%	2	1		1
Manguredjipa	3	2	66.7%				0
Masereka	15	6	40.0%				0
Musienene	94	50	53.2%	2	0		0
Mutwanga	2	2	100.0%				0
Oicha	9	6	66.7%	1	0		0
Vuhovi	7	4	57.1%				0
Haut-Uélé	253	106	41.9%	4	0	1	1
Boma							
Mangbetu	29	14	48.3%				0
Gombari	3	1	33.3%				0
Isiro	92	37	40.2%	0	0	1	1
Pawa	43	22	51.2%	3	0		0
Rungu	2	1	50.0%				0
Wamba	84	31	36.9%	1	0		0
Tshopo	22	9	40.9%	0	0	0	0
Bafwasende	2	0	0.0%				0
Kabondo	4	1	25.0%				0
Lubunga	1	0	0.0%				0
Makiso-Kisangani	10	5	50.0%				0
Mangobo	3	2	66.7%				0
Tshopo	1	0	0.0%				0
Wanie-Rukula	1	1	100.0%				0
South Kivu	3	1	33.3%	0	0	0	0
Miti-Murhesa	3	1	33.3%				0
Bas Uélé	4	3	75.0%	0	0	0	0
Buta	1	1	100.0%				0
Viadana	1	0	0.0%				0
Ganga	2	2	100.0%				0
Total	6686	3226	48.3%	82	36	15	51

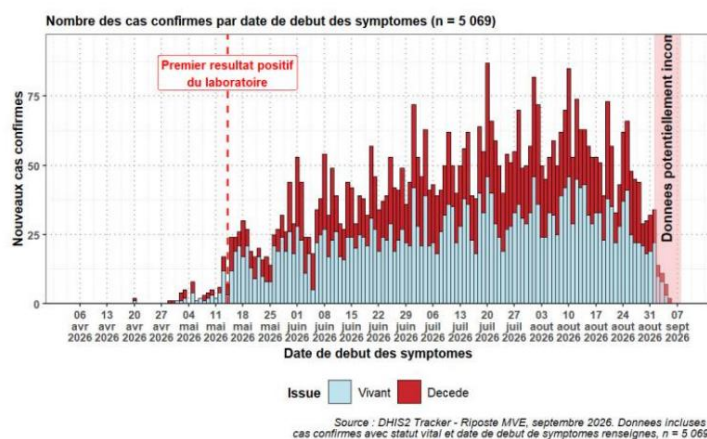
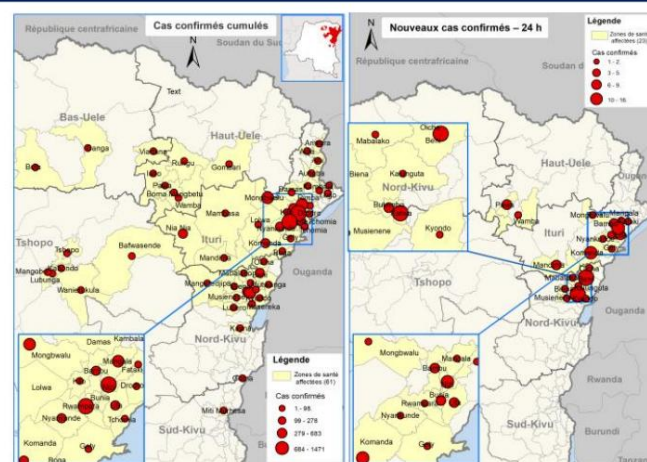
*These are the confirmed deaths that occurred in the various CTEs in the province and are being broken down to be classified by health zone.

NA: Not Applicable.

** These deaths were recorded in the various CTEs of the Ituri province and are currently being assessed.

Table 3. Status of alerts notified by province, as of September 6, 2026

DPS	Alerts Received		Total alerts	Alerts verified and validated (suspected case)		Alerts verified and invalidated (no suspected cases)		Number of suspects investigated	Number of suspects investigated and transferred to CT/CTE (of the day)
	Living	Deceased		Living	Deceased	Living	Deceased		
Haut-Uélé	5	2	7	5	2	0	0	7	4
Bas Uélé	15	0	15	4	0	11	0	4	0
Ituri	651	61	712	76	59	359	0	135	65
North Kivu	542	31	573	41	26	467	9	67	30
South Kivu	ND	ND	N / A	ND	ND	ND	ND	ND	ND
Tshopo	98	1	99	1	1	98	0	2	0
Total General	1311	95	1,406	127	88	935	9	215	99

Figure 1. Evolution of cases by date of symptom onset over the last 21 days**Figure 2. Spatial distribution of confirmed cases (24h and cumulative) by health zone.**

1. COORDINATION

1.1. Main Actions

- Monitoring and support of response actions in all affected provinces.

2. RESPONSE ACTIONS BY PILLAR

1.1.1. Epidemiological surveillance

1.1.2. Main Actions

Over the past 24 hours, a total of 1,406 alerts have been recorded in 5 Of the 6 affected provinces (South Kivu did not report), 1,159 (82.4%) were verified. All 215 alerts validated as suspected cases were investigated (100%) and 99 of them were transferred to the CT/CTE (Table 3). The daily proportion, at 85.3% (20,888 views out of 24,479 to follow), remained stable. compared to the previous day (85.7%), almost similar to the 85% threshold; 87.0% in Ituri (11,192/12,871), 85.4% in North Kivu (8,498/9,950), 70.4% in Bas Uélé (107/152), 76.1% in Haut Uélé (900/1,183) and 59.1% in Tshopo (191/323).

1.1.3. Challenges

Insufficient teams dedicated to surveillance activities and more specifically to contact tracing in the Bas Uélé province, although an improvement has been noted.

1.2. Update on checkpoint and point of entry (PoC/PoE) activities

1.2.1. Main actions

Four (4) alerts were notified to the various PoE/PoCs on September 6, 2026.

In Ituri, a body being transported from the morgue was intercepted at the Mudzibala Point of Control (Nzere Health Zone, Bunia Health Zone): the alert was notified, validated, and the body was sent to the morgue for processing. In North Kivu, a body being transported from Biakato to Beni was intercepted at the Kasitu Point of Control, notified, investigated, and sent to the morgue for processing.

In Bas-Uélé, two alerts were notified to the PoC PK3 Mboti (AS Mboti, ZS d'Ango); both were subsequently verified and invalidated.

Table 4. Status of key indicators for PoC/PoE by province, as of September 6, 2026

Indicators	Ituri	North Kivu	South Kivu	Tshopo	Haut-Uélé	Bas-Uélé	Overall
Report completeness	88.3% 100%	55.5% 88.8%	64.0% 34.6%	76.2%			
% of functional strategic PoE/PoC	9.1%	N / A	N / A	N / A	64.0%	N / A	36.4%
Number of people who have switched to PoE/PoC	128,390	89,946	7,820	9,534	12,709	4,283	252,682
% of travelers screened: 98.0% 98.2%	98.6% 99.0%	99.0% 98.6%	98.1%				
% of travelers who washed their hands	96.9% 98.2%	98.6% 99.0%	99.0% 98.6%	97.7%			
% of travelers made aware	99.1% 98.2%	100% 100%	99.0% 94.4%	98.9%			
Number of alerts notified	1	1	0	0	0	2	4
Number of live alerts validated (suspected cases)	0	0	0	0	0	0	0
Number of bodies recovered today	1	1	0	0	0	0	2
% of suspected cases transferred to CT/CTE	N / A	N / A	N / A	N / A	N / A	N / A	N / A
% of lifeless bodies swabbed/secured	100% 100%		N / A	N / A	N / A	N / A	100%
% of suspected cases investigated within 24 hours	100% 100%		N / A	N / A	N / A	N / A	100%

1.2.2. Challenges

Operationalization of PoE/PoC is suboptimal: completeness remains low in Bas-Uélé (34.6%, 9 reports received out of 26 expected), South Kivu (55.5%, 5/9), and Haut-Uélé (64.0%, 7/11), where four PoE/PoCs (Dusu, Apodo, Aungba, and Gossamu) failed to transmit data due to lack of internet connection. In Ituri, the Avakubi PoC remains non-functional due to insecurity, four PoCs (Nzebi, Galay, Pluto, and Bey) are on strike for non-payment, and two (Chamugamba and Pabiri) are unreachable; in Tshopo, only 18 of the 30 planned PoCs are activated.

1.3. Laboratory

1.3.1. Main actions

- **Ituri : 39 new positive results** (23 living and 16 deaths) out of 260 new samples received and analyzed (**positivity: 15.0%**).
- **North Kivu : 39 new positive results** (19 living and 20 dead) out of 170 samples analyzed (**positivity of 22.9%**).
- **Haut-Uélé: 4 new positive results** (4 live) out of 22 samples analyzed (**positivity: 18.2%**).
- **Bas-Uélé: 3 samples** received and tested (3 live), **all came back negative**.
- **Tshopo: 1 sample** received and tested (1 live), **it came back negative**.

1.4. Infection Prevention and Control (IPC/EDS)

1.4.1. Main actions

- **In Ituri**, 15 of the 69 planned infection control rings were opened (21.7%), 66 of the 66 identified households were decontaminated (100%), and 12 of the 13 health facilities were decontaminated (92.3%); 15 infection prevention and control (IPC) kits were distributed to households. Regarding the blood and control system (BCS), 82 alerts were recorded, including 3 postponements from the previous day, 59 BCS were carried out (72.0%), and 50 of the 59 bodies were swabbed (5 failures), with 18 postponements at the end of the day.
- **In North Kivu**, all 12 planned infection control rings were opened (100%), 7 out of 7 identified households were decontaminated (100%), and 11 out of 12 health establishments were decontaminated (91.7%), without the distribution of kits to households; 8 health facility assessments were carried out with an average score of 28% (3 in Beni, 4 in Musienene, and 1 in Manguredjipa), and 30 hygiene workers were trained in infection prevention and control/water, sanitation, and hygiene (IPC/WASH) in Kalunguta.
- **The EDS pillar received 51 alerts, including 22 postponements from the previous day, carried out 50 EDSs and swabbed the 51 bodies (100%).**
- **In Tshopo**, decontamination of the Matete Health Center (Mangobo Health Zone, 20th case), the KSJ Hotel (Umoja Health Center, Kabondo Health Zone, 21st case), the large meeting room, the corridor and 2 latrines of the Provincial Health Division as well as the Medical Training Center; provision of IPC supplies in 2 health centers in the Kabondo Health Zone and monitoring/support of 13 health centers in 5 health zones with the briefing of 122 providers on IPC measures; 1 ring opened and 1 public place decontaminated.

1.4.2. Challenges

- Weak implementation of IPC standards in Ituri: the opening of the rings remains very incomplete (21.7%, 15/69), decontamination teams remain insufficient in Fataki, Mangala, and Komanda, mobility is lacking in Bambu, Mandima, and Mangala, and Mandima has been deprived of activities due to a lack of IPC supplies; in Tshopo, insufficient IPC supplies in social and solidarity economy (SSE) facilities and untrained service providers; in Bas-Uélé, insufficient trained personnel and WASH infrastructure; in Haut-Uélé, IPC/DHS data were not transmitted. Regarding DHS, 5 bodies could not be swabbed in Ituri due to community resistance (Nia-Nia, Mandima), and insufficient transportation persists in Kilo, Nia-Nia, Mandima, and Fataki.

1.5. Holistic care

1.5.1. Main actions

- **In Ituri**, there were 55 new admissions to the various dedicated facilities, compared to 79 discharges, including 11 recoveries. Thus, 484 patients are hospitalized out of 978 beds, representing an occupancy rate of 49.5%.
- **In North Kivu**, 67 new admissions and 58 discharges were recorded, including 1 recovery; 262 patients are hospitalized expressing an overall bed occupancy rate in CTE/CT is in oversaturation (123.2%; 220 beds available).

- **In Haut Uélé**, there were 13 new admissions compared to 6 discharges (3 recovered). Thus, 68 patients are hospitalized for 128 beds, representing an occupancy rate of 50.8%.
- **In Tshopo**, no new admissions were recorded and two discharges (no recoveries). Five (5) patients are in isolation out of 25 beds, representing an occupancy rate of 20.0%.

1.5.2. Challenges

Low capacity and saturation: in North Kivu, the overall occupancy rate reaches 123.2% and some patients continue to be treated in non-standard structures; in Ituri, the CTE/CTs of Nizi, Mambasa and Mandima are saturated on the side of suspected cases, 39 patients are in critical condition and the extension work of Nizi and construction of Bambu remain in progress; in Bas-Uélé, the absence of CTE/CTs and ambulances limits the isolation of cases.

1.6. Vaccination

1.6.1. Main actions

Vaccination with the ERVEBO vaccine remains suspended in Bas-Uélé due to a stock shortage in Buta; data entry in DHIS2 continues. In Ituri, the plan to launch vaccination activities for people living with HIV/AIDS (PLHI) in the Bunia and Mongbwalu health zones has been validated with MSF support.

1.6.2. Challenges

Stock shortage of Ervebo in Buta leading to a temporary suspension of vaccination of PPL and TPL, lack of mobility for vaccination teams in advanced strategy and lack of micro vaccination plans for PPL in the 6 priority health zones of Bas-Uélé (Aketi, Ango, Bili, Buta, Ganga and Viadana).

1.7. Continuity of Care

In Bas-Uélé, post-exposure prophylaxis of high-risk contacts continued in the Ganga health zone, awareness-raising of 14 young people (8 men and 6 women) on the first suggestive signs of Ebola at ETS Paris Soir in Buta and evaluation of the site to house a transit center at the HGR of Buta.

In Bas-Uélé, 16 high-risk contacts remain under prophylaxis in the health zones of Buta (10) and Ganga (6), with no new admissions, discharges, or deaths during follow-up.

In Haut-Uélé, the Vice-Governor of the province unloaded and distributed kits to those who had recovered in the Wamba health zone, and issued recovery certificates for the Isiro and Wamba health zones. In Tshopo, 111 cases benefited from the experimental treatment, including 13 confirmed cases (3 on MPB134 and 10 on Remdesivir: 7 recovered, 3 deceased, 1 escaped, and 2 still receiving treatment) and 98 PEP (47 treatments completed, 9 interrupted, and 42 ongoing).

1.8. Risk communication and community engagement

1.8.1. Main actions

- **In Ituri**, the response focused on the latest confirmed cases within the last 48 hours: 110 people affected, including 41 women in Rwampara (Hoho, Shari and Dele, 9 cases), 102 people in Lita (Mutumbi, Katoto and Lita, 8 cases), and 25 people, including 15 women, in Nia-Nia (Alimasi, 2 cases). Results were communicated to families, contact tracing was supported, and households were prepared for decontamination. Officers from the 32nd Military Region of the FARDC were briefed at the Rwampara COVID-19 Treatment Center (BCZ) on preventive measures against Ebola virus disease (EVD), followed by a visit to the Rwampara Ebola Treatment Center (CTE).
- **In North Kivu**, 11,903 home visits reached 47,902 people and 46 educational talks reached 943 people; 7 mass communication campaigns reached 11,423 people in schools; 156 of the 450 expected alerts were reported by stakeholders

community (34%), 5 of the 8 feedbacks captured were resolved within 48 hours (62%), the 2 incidents recorded were resolved (100%) and 870 of the 4,750 people expected were engaged in community actions (18%).

- **In Tshopo**, 13,650 people were sensitized (7,508 women and 6,142 men), 317 leaflets were distributed, and 7 banners were displayed; 3 community feedback sessions were collected, and 2 rumors were clarified. In Haut-Uélé, a mass communication activity in the Wamba health zone reached 3,705 people (2,406 women and 1,299 men), 5 recoveries were celebrated, 12 radio programs were broadcast, and 52 people participated in educational discussions about cases and in health areas experiencing resistance.
- **In Bas-Uélé**, the MVE training was delivered to over 100 motorcycle taxi drivers, a joint briefing was held for 38 people in the Baruti health district, support was provided for the briefing of 51 community liaisons in the Buta health zone, 10 interactive programs and 36 educational spots were produced and broadcast on 12 partner community radio stations, and 6 alerts were raised by the community.

1.8.2. Challenges

Persistent community resistance: in Haut-Uélé, a confirmed escapee found in Nduka (Isiro Health Zone) categorically refused to return to the CTE, her husband threatening to commit suicide, and 22 contacts were listed along her route, while rumors persist about the origin of the virus and a shortage of posters and leaflets is reported; in Tshopo, resistance to investigations, sampling, isolation and transfer to the CTE, with 10 contacts refusing follow-up in the Matete Health Area (Mangobo Health Zone); in Bas-Uélé, impassable roads and no mobile network in several villages.

1.9. Mental Health and Psychosocial Support (MHPSS)

1.9.1. Main actions

- **In Ituri**, 22 new confirmed cases and 23 of the 130 suspected cases benefited from SMSPS interventions; 59 test results were announced, including 22 positive results; 44 people receiving support were assisted; 51 households received support; and 14 recoveries were monitored in the community. In **Haut-Uélé**, communication efforts surrounding the 5 most recent confirmed cases reached 71 households in Isiro, Pawa, and Wamba.
- **In Haut-Uélé**, 2 new confirmed cases and 43 confirmed cases under follow-up benefited from SMSPS interventions, as well as 3 new suspects and 1 suspect under follow-up; 15 announcements of results including 2 positive; 96 accompanying persons and 79 PPL supported as well as 4 family members.

1.9.2. Challenges

In Ituri, there is low coverage of SMSPS interventions around the latest confirmed cases in the Bunia, Nizi and Komanda health zones.

1.10. Logistics

1.10.1. Main actions

- **In Ituri**, 16 of the 32 organized patient transfers were successful (50%) and 19 patients were evacuated; food support was provided to 450 of the 467 hospitalized patients (96%), 17 children under LNPE/SLM at the CTE (100%), 2 in nursery and 25 in the community; 5 ambulances are immobilized due to a lack of hygienists and nurses (1 in Bambu and 4 in Nizi) and food needs remain in Kilo (3), Lolwa (5), Mambasa (5) and Mandima (4).
- **In North Kivu**, 13 calls were received, resulting in 13 completed missions, 12 of which were successful (92%), and 9 ambulances were mobilized out of 11 available (82%); 40 of the 40 patients (100%) and 7 accompanying persons received a hot meal, 3 confirmed unstable patients received a specific diet at the Beni Ebola Treatment Center, and 52 frontline providers received meals.

- In Bas-Uélé, 56 patients, 60 accompanying persons and 25 PPLs benefited from three hot meals, with the exception of the ZS of Boma Mangbetu, Rungu and Gombari, and a vehicle was given to the laboratory pillar by the coordination of Isiro.

1.10.2. Security

1.10.3. Main actions

- In North Kivu, three activities to maintain public discipline were carried out in the Beni sub-coordination: accompaniment and security coverage of the PCI team at the PIDS dispensary (Mangote Health Center) and at the CEBECE Hospital (Kasanga Health Center) for decontamination, as well as of the EDS team for the burial of a confirmed case at the Masiani public cemetery; all these activities were covered at 100%.

1.10.4. Challenges

In North Kivu, there is a lack of a permanent vehicle for the security team and a lack of financial incentive for the agents; there is a need to systematically inform the security team before any field visit.

1.11. PSEA (Prevention of Sexual Exploitation and Abuse)

1.11.1. Main actions. • In Ituri, a

briefing was held for 36 people, including 26 PRSEAH focal points from the 13 health zones, on Ebola virus disease (EVD), adverse events following encephalopathy (AEFI), zero tolerance, and reporting mechanisms. The toll-free numbers 495555 and 151 were disseminated, and leaflets were distributed, with support from the WHO. In Bas-Uélé, a briefing was held for 36 people, including 26 PRSEAH focal points from the 13 health zones, on EVD, AEFI, zero tolerance, and reporting mechanisms. The toll-free numbers 495555 and 151 were disseminated, and leaflets were distributed, with support from the WHO.

1.11.2. Challenges

- Still insufficient coverage of PSEA activities, with only 13 of the 36 health zones in Ituri (36.1%) aware of the reporting mechanism; lack of mapping of stakeholders and refusal of some partners to align; insufficient mass awareness materials; lack of computer kits and office supplies for the pillar.

KEY MESSAGES	TIMELINE OF KEY EVENTS
• Consolidated encouraging progress in epidemiological investigations (100% of suspected cases investigated) and a stability in the daily proportion of contact tracing (85.3%) roughly similar to the threshold of 85.0%. • Continued reporting in the provinces of Ituri, North Kivu and Haut-Uélé highlights sustained community transmission of the disease). • As of September 6, 2026, 82 new confirmed cases, including 36 community deaths and 15 intra-CTE deaths, were recorded in the last 24 hours, bringing the national total to 6,686 confirmed cases and 3,226 death (case fatality rate: 48.3%). • Ituri remains the epicenter of the epidemic, accounting for 80.2 % of cumulative confirmed cases and 47.6% of new cases reported in the last 24 hours, tied with North Kivu province. • Strengthening priority interventions (surveillance, care, IPC, logistics and community engagement) remains essential to quickly interrupt transmission chains.	• From April 24 to May 15, 2026, the epidemic progressed from the detection of the first suspected cases to the biological confirmation of the Ebola Bundibugyo virus, leading to the official declaration of the 17th Ebola Virus Disease epidemic in the Democratic Republic of Congo. • Between May 17 and 18, 2026, national authorities, with the support of partners, strengthened the response by declaring a public health emergency of national and then continental scope, accompanied by the mobilization of funds.



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